A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

27 May 2026

Dr Robin Wong
Turn The Corner

Dear Dr Wong,

Re: Mrs Jean James
DOB: 07/06/1945 Phone: 0447 407 003
Address: 158 Elm Street, NORTHCOTE, VIC, 3070
Email: jvj15876@gmail.com

Summary

- **Gastric ulcers on gastroscopy with biopsies negative for Helicobacter pylori and malignancy.**
- **Hyperplastic colonic polyp and colonic angioectasia.**
- **Repeat gastroscopy required in July to confirm ulcer healing.**
- **Ongoing esomeprazole 20mg BD before meals.**
- **No action required by you.**

I had the pleasure of conducting a review with Jean following her gastroscopy and colonoscopy, which were performed to investigate iron deficiency anaemia.

The gastroscopy revealed the presence of gastric ulcers. I performed several biopsies to exclude unusual causes of ulceration, including Helicobacter pylori, and I am pleased to report that these were negative. Crucially, there was no evidence of malignancy on the histological samples. We discussed that while her aspirin use is necessary for her vascular health, it is a likely contributor to this ulceration, particularly in the context of her smoking history.

The colonoscopy did not reveal any significant pathology. I removed a small polyp which was histologically hyperplastic; therefore, it is not precancerous and she does not require a surveillance colonoscopy. I did identify colonic angioectasia, which may be an additional contributor to her low iron levels due to occult blood loss. At this stage, I do not believe endoscopic intervention or cauterisation of these vascular malformations is necessary, but it remains an option should her iron stores be difficult to maintain.

Impression and Plan I have asked Jean to continue taking esomeprazole 20mg twice daily, ideally before meals, and I have provided her with a new prescription. Given the finding of gastric ulcers, a follow-up gastroscopy is mandatory to confirm complete healing and to definitively exclude an underlying early gastric cancer that may not have been captured on initial biopsies. I have advised this be performed from mid-July onwards.

In the long term, Jean will likely require maintenance proton pump inhibitor therapy, potentially at a reduced dose of 20mg daily, given her ongoing need for aspirin and her current smoking status. I will review her again following her surveillance gastroscopy.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

Investigation Results:

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B012474

CLINICAL NOTES

Iron def. constipation. Gas body, random colon,
duo, antrum = ulcer + gastritis -rule out dysplasia, rule in Helicobacter. Sigmoid polyps.

MACROSCOPIC DESCRIPTION

1. Specimen labelled "Gastric body BX".

Five mucosal fragments measuring 2-4 mm. A1.

2. Specimen labelled "Antrum BX".

Four mucosal fragments measuring 3-4 mm. A1.

3. Specimen labelled "Duodenal BX".

Two mucosal fragments measuring 3-10 mm. A1.

4. Specimen labelled "Random colon BX".

Three mucosal fragments measuring 2-3 mm. A1.

5. Specimen labelled "Sigmoid polyps".

Four tan polypoid fragments measuring 15 x 4 x 2 mm in aggregate. Largest two fragments 2 LS each, remaining fragments processed whole.A1.

MICROSCOPIC DESCRIPTION And DIAGNOSIS

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1. Gastric body - Gastric body mucosa with no erosion or acute inflammation. There is mild chronic gastritis.

No atrophy, metaplasia or dysplasia.

No Helicobacter seen.

2. Antrum biopsy - Gastric antral mucosa with no erosion or ulceration. There is no inflammatory cell infiltrate.

There is no metaplasia or dysplasia.

No Helicobacter are seen.

3. Duodenal biopsy - Normal villous architecture with no inflammation or erosion. No evidence of coeliac disease or parasites.

4. Random colon - Normal crypt architecture. No erosion. No inflammatory infiltrate. No thickening of the collagen plate. No atypia.

5. Sigmoid polyps - Hyperplastic polyps.

Pathologist: Dr Melissa Robbie

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